

Bug Report & UX Issue Log

Complete Field QA Session — April 2026

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Project: EaseHealth — Google.org Grant n316841
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Sessions covered: Both QA sessions — all issues from this report cycle

Summary

17 issues identified across two consecutive QA sessions covering the full CHW user journey from app open through case history review. Issues span crash-level bugs confirmed in field deployment, data integrity gaps affecting pilot metrics, and UX friction that will impact CHW adoption. Critical and High issues must be resolved before the May 2026 CHW pilot deployment.



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Issue	Area / Screen	Severity
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#16 Referral page triggered after tapping Save on the outcome screen — wrong point in flow	Confirm Outcome / Save Assessment	MEDIUM
#17 No app timeout or shift-change identity management between CHWs sharing a device	System-wide	LOW / ENHANCEMENT

Deployment readiness: Issues #01, #02, and #03 were observed in active field conditions and represent the highest risk to the May pilot launch. These three must be resolved and re-tested before any further CHW onboarding.

Critical Issues

These three issues were observed or confirmed in active field conditions. They represent device failure risk, data loss, and app crashes. None of these are acceptable at pilot launch.

#01 Screen stays on indefinitely — no display timeout	
Screen	System-wide
Severity	CRITICAL
Type	Bug / Power Management
Description	After opening the app, the phone screen never times out. The display remains on until the battery is completely drained or the user manually locks the device. This was confirmed in the field across all devices where EaseHealth was installed.
Steps to Reproduce	Open EaseHealth on any device. Leave it idle at any screen. Observe: screen never turns off regardless of the device system timeout setting.
Expected Behaviour	The app should release the wake lock when idle and respect the device's native screen timeout. The screen should only stay on during active model inference or active CHW input — not at rest states like the Home screen or post-assessment screens.
Recommendation	• Release the WakeLock when the app is idle for more than 5 minutes or moves to the background. • Hold WakeLock only during active consultation sessions and model inference — not on the Home screen or Consultation Saved screen. • Test with Android battery optimisation enabled to confirm the app does not bypass system power management. • Consider a visual idle timeout warning ('Still with a patient?') before screen lock to prevent accidental session loss.
Data / Pilot Impact	Field critical: CHW devices operate off-grid. Battery drainage directly impacts operational readiness during the pilot.

#02 Screen remains interactive during model inference — causes crashes	
Screen	Assessment / Symptom Entry
Severity	CRITICAL
Type	Bug / Race Condition
Description	After tapping 'Generate Assessment', the button label changes to 'Analyzing...' but the symptom entry screen remains fully interactive. CHWs can continue adding or removing symptoms while the model is running. This has caused the app to crash or become stuck in a permanent analyzing state, confirmed in the field.
Steps to Reproduce	Enter symptoms on the assessment screen. Tap 'Generate Assessment'. While 'Analyzing...' is displayed, tap symptom fields and add more inputs. Observe: app crashes or hangs indefinitely.

Expected Behaviour	The entire screen must be locked to user input the moment inference begins. A full-screen overlay or disabled state should block all touch events — not just the generate button.
Recommendation	- Implement a full-screen semi-transparent overlay during model inference that intercepts all touch events. - Display a visible loading spinner with clear copy: 'Analysing case — please wait...' - If state changes are detected mid-inference, cancel the request and surface a recoverable error rather than allowing a corrupted state. - Add a 60-second inference timeout with a retry prompt if the model does not return a result.
Data / Pilot Impact	App crashes result in lost consultation data and erode CHW trust in the tool before the pilot formally begins.

#03 Return to Home silently discards the active assessment	
Screen	Assessment Screen
Severity	CRITICAL
Type	Bug / Data Loss
Description	Tapping 'Return to Home' during or after an assessment immediately navigates to the home screen and silently discards the entire consultation — no confirmation modal, no save prompt, no warning. The save confirmation modal that exists for certain other navigation paths is not triggered by this action.
Steps to Reproduce	Start a new assessment. Enter patient data and symptoms. Tap 'Return to Home'. Observe: the app navigates to home with no prompt and the assessment is gone.
Expected Behaviour	Every navigation path that exits an active assessment must trigger the same save/discard confirmation modal — including Return to Home, hardware back button, and back gesture swipes.
Recommendation	- Intercept all exit navigation paths (Return to Home, hardware back, gesture swipe back) with the existing confirmation modal. - Modal copy should be: 'Do you want to save this assessment?' with options 'Save Assessment' and 'Discard'. - Log abandoned assessments as a data event even when discarded — session duration and symptom count for abandoned consults has operational value. - Audit every screen in the app for unguarded exit paths and apply the same interception consistently.
Data / Pilot Impact	Silent data loss. Completed consultations are dropped with no record, creating gaps in surveillance data and case history.

High Priority Issues

These issues directly affect data integrity, pilot metric accuracy, or clinical data completeness. Resolve before or during the first week of CHW deployment.

#04 Patient sex field removed — regression from previous version	
Screen	Assessment / Symptom Entry
Severity	HIGH
Type	Regression / Missing Field
Description	The patient sex field (Male / Female) was present in the previous build of EaseHealth but has been removed in the current version. Sex is a required field in the surveillance data schema and is needed for IDSR disaggregation and Google.org pilot reporting.
Steps to Reproduce	Open a new assessment. Review all available input fields. The sex field is absent.
Expected Behaviour	Sex (M / F) must be a required field on the symptom entry screen, positioned near the age group selector.
Recommendation	- Re-add sex as a required binary field — segmented control or radio (Male / Female). - Map it to the 'sex' column in the consultations table (CHAR(1): 'M' or 'F'). - Make it a required field that must be selected before 'Generate Assessment' is enabled. - Confirm it syncs correctly to the backend and appears in the IDSR weekly summary view.
Data / Pilot Impact	Sex disaggregation is required for IDSR reporting and pilot outcome metrics. Missing data compromises dataset integrity for the Google.org deliverable.

#05 View Case History button missing badge and icon on Consultation Saved screen (first render)	
Screen	Case Saved Locally → Continue → Consultation Saved
Severity	HIGH
Type	Bug / UI Rendering
Description	Full flow: After saving or referring a patient for the first time, the app navigates to a 'Case Saved Locally' screen with a Continue button. When the user taps Continue, they arrive at the Consultation Saved page which shows two buttons: 'Start New Assessment' and 'View Case History'. On this first render, the View Case History button is missing its cases count badge and its left-side icon. Both elements only appear after the user taps the button, navigates to Case History, and returns. This is a state initialisation issue — the component mounts before the case count has been loaded.

Steps to Reproduce	Complete an assessment. Refer or save the patient. Land on 'Case Saved Locally' screen. Tap Continue. On the Consultation Saved page, observe 'View Case History' button: no icon visible, no count badge. Tap it, go to Case History, then press Back. The badge and icon now appear correctly.
Expected Behaviour	The View Case History button should render with the correct updated case count and its icon on the first render immediately after a consultation is saved.
Recommendation	- Pre-fetch or pass the updated case count as a navigation parameter when routing to the Consultation Saved screen so the component has data before it mounts. - Ensure the case count state is not lazily initialised on navigation — it should be ready at mount time. - Use a loading skeleton state for the badge if data must be fetched asynchronously, rather than rendering an empty/broken state.

#06 Referral destination and reason not shown in Case History detail

Screen	Case History / Case Detail
Severity	HIGH
Type	Missing Feature / Data Gap
Description	When a patient is referred, the Case History detail view displays only a 'Referred' badge. The specific facility or level the patient was referred to and the clinical reason for referral are not recorded or displayed. Since there is no unified referral alert system yet, the case history is the only audit trail for referral decisions.
Steps to Reproduce	Complete an assessment and refer the patient to a facility. Navigate to Case History. Open the case detail. Referral is shown only as a status badge — no destination, no reason.
Expected Behaviour	The case detail view should display a dedicated 'Referral Record' section containing: (1) Referred to — facility name and level (e.g. 'Kakiri HC III'), (2) Reason for referral — primary classification or danger signs, (3) Referral timestamp.
Recommendation	- Capture 'referral_facility', 'referral_level', and 'referral_reason' fields at the point of referral confirmation. - Persist these fields with the consultation record and surface them in the case detail view as a distinct 'Referral Record' section. - Do not rely solely on the status badge for referral documentation — the badge is a UI indicator, not a record. - These fields also feed the referral escalation rate metric which is a tracked pilot KPI.
Data / Pilot Impact	No audit trail for referral decisions. Referral record-keeping is the only source of this data before a unified referral alert system is built.

#07 Session duration timestamp is inaccurate — starts at first keystroke, not consultation open

Screen	System-wide / Analytics
Severity	HIGH
Type	Bug / Data Integrity
Description	The app records session duration starting from when the CHW first types in a symptom field, not from when the new consultation is opened. Pre-consultation time — reading the interface, understanding the patient, setting up — is not captured. The 'session_duration_seconds' field in the surveillance schema therefore understates actual consultation length.
Steps to Reproduce	Open app. Start a new assessment. Wait 30 seconds before typing anything. Begin entering symptoms. Observed session duration starts from first keystroke, not from consultation open.
Expected Behaviour	Session duration should be recorded from the moment the CHW opens a new consultation, not from first input event.
Recommendation	• Initialise the session timer on the 'Start New Assessment' action, not on the first input event. • Consider capturing both: 'session_start' (new assessment opened) and 'first_input_at' (first symptom entered). Both are analytically useful. • Verify the timer persists correctly through offline mode and device sleep/wake cycles. • This is a tracked pilot metric for CHW workload analysis — accuracy matters for the Google.org report.
Data / Pilot Impact	Inaccurate session duration skews the 'time per consultation' pilot KPI and CHW workload analysis in the final Google.org report.

Medium Priority Issues

UX friction, copy misalignments, and behavioural inconsistencies. These degrade CHW experience and should be resolved before or during the pilot where development capacity allows.

#08 'New Assessment' button does not trigger save confirmation modal	
Screen	Assessment Screen
Severity	MEDIUM
Type	Bug / Inconsistent Behaviour
Description	The save confirmation modal fires correctly on some exit paths but is not triggered by the 'New Assessment' button. A CHW who taps 'New' mid-consultation will silently lose their current assessment. The existing modal logic for other paths is correct — it is simply not applied to this navigation trigger.
Steps to Reproduce	Begin an assessment and enter symptoms. Tap the 'New Assessment' button. Observe: no confirmation modal, current assessment is silently discarded.
Expected Behaviour	Tapping 'New Assessment' during an active consultation must trigger the same save confirmation modal as all other exit paths.
Recommendation	• Apply the confirmation modal to the 'New Assessment' button path, consistent with all other exit paths. • Modal options: 'Save Assessment' and 'Discard and Start New' — not ambiguous Yes/No. • Audit all navigation triggers in the app that can exit an active assessment and confirm each one is covered.

#09 Image input does not inform CHW to pre-capture the image	
Screen	Assessment / Image Input
Severity	MEDIUM
Type	UX Issue / Missing Guidance
Description	When a CHW taps 'Add Image', the flow opens a file picker that requires the image to already exist on the device. There is no instruction telling the CHW to capture the image with their camera app before starting the assessment. CHWs will expect in-app camera access and encounter a dead end.
Steps to Reproduce	Tap 'Add Image' on the assessment screen. App opens a file picker. No camera option is available. CHW has no image to select and receives no guidance.
Expected Behaviour	Before presenting the file picker, the app should display a clear instruction: 'Capture your photo with the camera app first, then return here to attach it.' This should appear as a persistent helper label or first-time tooltip.
Recommendation	• Add persistent helper text below the 'Add Image' button explaining the pre-capture requirement.

	- Consider a one-time instructional modal on the first use of the image feature. - Longer-term: enable in-app camera capture directly to remove this friction point before scaled deployment.
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#10 Voice note does not clarify it is English transcription only — no translation

Screen	Assessment / Voice Input
Severity	MEDIUM
Type	UX Issue / Missing Guidance
Description	The voice input feature transcribes audio to text but does not translate from local languages (Luganda, Runyankole, etc.) to English. There is no indication of this limitation in the UI. CHWs who primarily speak local languages may use voice input expecting translation and receive garbled or empty transcriptions.
Steps to Reproduce	Tap the voice note button. Record in a local language such as Luganda. Output is garbled or incomplete with no explanation of why.
Expected Behaviour	A clear persistent label should appear beneath the voice input button: 'English only — transcribes speech, does not translate.'
Recommendation	- Add a static helper label directly below the voice note button stating the English-only limitation. - Consider an onboarding tooltip on first use before the CHW records, not after they get a bad result. - Flag local language voice support as a named future enhancement — relevant to pilot learning for Karla and the scaled investment narrative.

#11 Two simultaneous toasts on assessment screen with the same message

Screen	Assessment Screen
Severity	MEDIUM
Type	UX Issue / Cognitive Load
Description	After saving an assessment, the screen shows two persistent toasts simultaneously: 'Assessment saved on this device' at the top and 'Case saved locally. Will sync when connected.' at the bottom. Both convey the same information in different words. Stacked persistent toasts create unnecessary cognitive load, particularly for CHWs in high-pressure field environments.
Steps to Reproduce	Complete and save an assessment. Observe two toast notifications appearing simultaneously on screen — one at the top and one at the bottom.
Expected Behaviour	One toast only. Combined message: 'Case saved. Will sync when online.' Auto-dismiss after 15 seconds.
Recommendation	Consolidate to a single toast: 'Case saved. Will sync when online.' — this covers both messages clearly.

	• Auto-dismiss after 15 seconds. • If the two toasts serve genuinely distinct functions, only show the offline-specific one when the device is offline. Never display both simultaneously under any condition.
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#12 Rename 'Signs and Symptoms' to 'Danger Signs'

Screen	Assessment Screen
Severity	MEDIUM
Type	Copy / Clinical Terminology
Description	The section label 'Signs and Symptoms' does not align with IMCI or UCG 2023 clinical terminology. 'Danger Signs' is the correct term used in Uganda Clinical Guidelines 2023 and in CHW training materials. Misaligned terminology creates confusion for clinically trained users and supervisors reviewing the tool.
Steps to Reproduce	Open a new assessment. Observe the section heading 'Signs and Symptoms'.
Expected Behaviour	Section heading should read 'Danger Signs' to align with UCG 2023 clinical language.
Recommendation	• Update the label from 'Signs and Symptoms' to 'Danger Signs'. • Audit all other screens and labels for non-UCG terminology and align them consistently. • Ensure IDSR syndrome codes and classification labels in the backend also use standard UCG/IDSR naming.

#13 Rename 'Confirm Outcome' to 'Save Assessment'

Screen	Confirm Outcome Screen
Severity	MEDIUM
Type	Copy / UX Clarity
Description	'Confirm Outcome' reads as a clinical validation step rather than a save action. CHWs completing a consultation need language that clearly signals they are finalising and saving their work — not making a clinical judgment call.
Steps to Reproduce	Complete an assessment and proceed to the outcome screen. The heading reads 'Confirm Outcome'.
Expected Behaviour	The screen heading should read 'Save Assessment' and all CTA button copy on this screen should use consistent save-oriented language.
Recommendation	• Rename the screen heading from 'Confirm Outcome' to 'Save Assessment'. • Review all button labels on this screen for copy consistency. • Ensure the confirmation modal triggered from other screens also uses 'Save Assessment' language.

#14 Case History cards — only the 'View' button is tappable, not the full card

Screen	Case History
Severity	MEDIUM
Type	UX Issue / Touch Target
Description	In Case History, only the 'View' button within a case card responds to taps. The card body — which is the visually dominant element — is not interactive. This creates a small touch target that will cause miss-taps on mobile, particularly for CHWs in field conditions using the device one-handed.
Steps to Reproduce	Navigate to Case History. Tap anywhere on a case card body (not the View button). Nothing happens.
Expected Behaviour	The entire card surface should be tappable and navigate to the case detail view.
Recommendation	• Wrap the full card component in a Pressable/Touchable element. • The 'View' button can remain as a visual affordance but the tap event should fire from the full card. • Ensure the tap target meets accessibility minimums (48x48dp minimum).

#15 Case detail drawer for #CASEDD3910 scrolls erratically and auto-dismisses

Screen	Case History / Case Detail Drawer
Severity	MEDIUM
Type	Bug / UI Rendering
Description	When viewing case #CASEDD3910 and attempting to scroll within the case detail drawer, the drawer animates erratically — rapidly bouncing up and down — until it dismisses itself. The issue appears isolated to this case, suggesting either an edge case in the data (unusually long content, null fields, unusual characters) or a scroll handler conflict specific to this record.
Steps to Reproduce	Navigate to Case History. Open case #CASEDD3910. Attempt to scroll within the detail drawer. Observe: drawer vibrates rapidly up and down and then dismisses itself.
Expected Behaviour	The drawer should scroll smoothly and remain stable regardless of content length or data edge cases.
Recommendation	• Inspect the raw data for #CASEDD3910 for edge case content — very long text strings, null values, or unusual characters that may be triggering a layout loop. • Check for conflicting scroll event handlers between the bottom sheet gesture recogniser and the inner ScrollView. • Test the drawer with other cases that have similar content length to isolate whether this is data-driven or a component bug. • Add a maximum content height constraint to the drawer to prevent layout overflow scenarios.

#16 Referral page triggered after tapping Save on the outcome screen — wrong point in flow

Screen	Confirm Outcome / Save Assessment
Severity	MEDIUM
Type	UX Flow Issue
Description	When a CHW selects 'Referred' on the Confirm Outcome screen and taps Save, a referral details page unexpectedly appears. The Save action should be terminal — a single committed tap that finalises the consultation and navigates to the Consultation Saved screen. Surfacing a new screen after Save breaks the user's mental model of that action.
Steps to Reproduce	Navigate to Confirm Outcome screen. Select 'Referred'. Tap Save. A referral details page appears unexpectedly after the save action.
Expected Behaviour	Referral destination and reason should be captured inline on the Confirm Outcome screen before the CHW taps Save — not triggered as a new screen post-save. The Save tap should be the last action in the flow.
Recommendation	• Conditionally reveal referral destination and reason fields inline on the Confirm Outcome screen when 'Referred' is selected (expand-in-place pattern). • Remove the post-save referral modal entirely. • The Save action must be terminal: one tap, data committed, navigate to Consultation Saved screen.

Low Priority / Enhancement

Structural improvements that do not block the pilot but should be scoped into the next sprint or the scaled deployment phase. The shift-change issue in particular has pilot metric implications and should be resolved before data analysis begins.

#17 No app timeout or shift-change identity management between CHWs sharing a device	
Screen	System-wide
Severity	LOW / ENHANCEMENT
Type	Enhancement / Identity Management
Description	EaseHealth uses role selection rather than login to identify the active user. There is no idle timeout and no mechanism to detect when a different person takes over the same device at a shift change. If two CHWs are in the same role and share a device, all subsequent consultations continue to be attributed to the first CHW's session — making per-CHW analytics unreliable. The question of whether to persist or clear data from the previous user on a handoff also has no current answer.
Steps to Reproduce	CHW A uses app on a shared device. Shift changes. CHW B picks up the device in the same role. All consultations are attributed to CHW A's ongoing session with no prompt or indication.
Expected Behaviour	The app should idle-timeout to role re-confirmation after a period of inactivity, and provide a visible 'End Shift' or 'Switch User' option on the home screen for deliberate handoffs.
Recommendation	• Implement a 30-minute inactivity timeout that returns the app to the role selection screen and prompts: 'Still on shift as [Role]?' • Add a visible 'End Shift' option on the Home screen for deliberate handoffs. • Define and document the data retention policy for shift handoffs: should open cases from the previous session be visible to the incoming CHW or not? • Longer-term: a 4-digit CHW PIN would solve attribution definitively without requiring a full login system, and would enable accurate per-CHW analytics for the pilot.
Data / Pilot Impact	CHW-level adoption rate and queries-per-CHW-per-week are tracked pilot KPIs. Unresolved session attribution makes both metrics unreliable.

Ease Health App — Bug Report

Prepared for: Field Team

Date: April 13, 2026

Status: Open — Awaiting Triage

Critical (P0)

BUG-01 — Data Persistence Failure

Assessment data is not persisted to local storage between sessions. On app restart, all in-progress and completed assessment data is lost. Expected behavior: data should survive app close/reopen regardless of sync status.

BUG-02 — Sync State Mismatch

The app incorrectly displays an offline sync warning (“will sync when connected”) while the device has an active network connection. Network state detection is returning a false negative, causing the app to queue data locally instead of syncing in real time.

BUG-03 — Duplicate Patient State on Pause

When a patient is paused, the record appears in both the active assessments list and the paused list simultaneously. The app is writing two separate state entries instead of transitioning the record from one state to the other.

High (P1)

BUG-04 — Camera Input Unavailable on Image Assessment

When the user selects the image assessment option, only file upload is offered. The device camera is not invoked as an input option. Expected behavior: user should be presented with both “Take Photo” and “Upload from Gallery” options.

BUG-05 — Invalid Input Returns Clinical Output

When a non-clinical image is uploaded (e.g. a table, screenshot, or document), the model returns structured clinical recommendations instead of an input validation error. Expected behavior: the app should detect non-clinical inputs and return a user-friendly rejection message such as “This image doesn’t appear to be a valid clinical input. Please upload a photo of the patient or a relevant clinical image.”

BUG-06 — Unsaved Assessment Not Gated Before New Assessment

A user can initiate a new assessment while a current assessment is unsaved or in progress, with no warning. Expected behavior: a modal should intercept this action and present two options — “Save current assessment and start new” or “Pause current assessment and start new” — based on the current state of the in-progress session.

BUG-07 — Referral Flow Triggered Incorrectly on Clinician Confirmation Page

On the clinician confirmation screen, selecting “Referred” as an outcome navigates the user into the referral creation flow. This page is intended to log what action was already taken, not to initiate a new referral. Selecting “Referred” should record the outcome and remain on the confirmation screen.

Medium (P2)

BUG-08 — Action Buttons Obstruct Assessment Content

The “Confirm Outcome,” “Refer Patient,” and “Pause Patient” buttons are persistently visible and consume significant screen real estate, blocking the assessment content below. Expected behavior: buttons should be hidden by default and revealed when the user scrolls to the bottom of the assessment, or collapsed into a sticky footer that expands on interaction.

BUG-09 — Keyboard Does Not Dismiss on Checkbox Interaction

When the user transitions from a text input field to tapping checkboxes (e.g. in the Danger Signs section), the soft keyboard remains on screen. Expected behavior: the keyboard should dismiss automatically when focus moves to a non-text interactive element.

BUG-10 — UI Jank on Saved Case View

The interface exhibits frame drops and visual instability (flickering/shaking) when navigating to and rendering a saved case. Likely caused by unoptimized list rendering or state re-renders on the saved cases screen. Recommend profiling with the device’s performance monitor to identify the offending component.

BUG-11 — Image Assessment Response Latency Exceeds 4 Minutes

The image assessment pipeline takes upward of 4 minutes to return results to the user, with no progress indicator shown during processing. Expected behavior: response time should be optimized and a loading state (progress bar or animated indicator) should be shown throughout. Anything beyond 60 seconds should surface an explicit wait message.

Low (P3)

BUG-12 — Inconsistent Model Response Quality

The model produces inconsistent clinical outputs for equivalent inputs. Example: entering “fever/flu” returns a response directing the user to consult the UCG 2023 guideline rather than generating a structured clinical assessment. This suggests the prompt handling or input normalization layer is not consistently routing inputs to the correct assessment flow.

Total Issues: 12

P0: 3 | P1: 4 | P2: 4 | P3: 1